

Postpartum - Vaginal Bleeding and Lochia - Female (Child)

Algorithm 1444 | Child (12-17 years) | Gender at birth: Female

Definition

- Postpartum vaginal bleeding assessment using published urgent maternal warning signs and postpartum haemorrhage guidance; remote pad counts are supporting observations, not a stand-alone diagnostic threshold.
- Child patient.
- Female.

Initial assessment

1. [DURATION] How many days/weeks since delivery?
2. [OPEN_TEXT] How many pads are being soaked per hour, and are there clots?
3. [YES_NO] Any dizziness, fainting, weakness, confusion, clammy skin, or a fast-beating heart?
4. [YES_NO] If the patient is under 18, can they speak privately now, and do they say they feel safe with the accompanying adult?
5. [CAREGIVER_OBSERVATION] What has a safe parent or caregiver observed about alertness, breathing, pain, swelling, fever, eating or drinking, sleep, play, and normal activity?

Triage and redirect questions

1. Redirect → Fainting

Was there a brief faint with full recovery, and is fainting now the primary concern?

A fully recovered brief faint belongs in the Fainting pathway.

1. Disposition level 100

Is there heavy or gushing vaginal bleeding, bleeding rapidly soaking pads, large clots, fainting or severe dizziness, weakness, confusion, pale or clammy skin, a fast-beating heart, or rapidly increasing bleeding?

These are published postpartum haemorrhage and urgent maternal warning signs. Postpartum haemorrhage usually occurs within 24 hours but can occur up to 12 weeks after birth.

1. Disposition level 70

Does the vaginal discharge have a foul smell, or has bleeding that was decreasing suddenly become heavier again?

A foul smell can indicate infection, and bleeding becoming heavier after decreasing needs prompt evaluation.

1. Disposition level 15

Is this typical lochia - gradually lightening and changing from red to pink/brown to white/yellow over several weeks - with none of the features above?

Normal lochia gradually decreases in amount and changes color over about 4-6 weeks after birth.

Advice

Emergency postpartum hemorrhage precautions

- Call Qatar emergency services on 999 now. Keep the patient lying down if safe, do not let them drive, and tell the call handler that they gave birth recently. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: worsening dizziness; worsening bleeding.

Urgent postpartum bleeding review

- Arrange prompt clinical assessment; the exact non-emergency Qatar maternity destination remains GOVERNANCE_REQUIRED.
- Call Back If: bleeding increases; fever develops.

Home monitoring for normal lochia

- Use sanitary pads (not tampons), and expect the discharge to gradually lighten in amount and change from red to pink/brown to white/yellow over about 4-6 weeks. Rest and stay hydrated.
- Call Back If: bleeding becomes heavier instead of lighter; foul smell, fever, or dizziness develops.

First aid

- Call Qatar emergency services on 999 now. Keep the patient lying down if safe, do not let them drive, and tell the call handler that they gave birth recently. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- ACOG, "3 Conditions to Watch for After Childbirth" (postpartum haemorrhage can occur up to 12 weeks after birth), <https://www.acog.org/womens-health/experts-and-stories/the-latest/3-conditions-to-watch-for-after-childbirth>
- CDC Hear Her urgent maternal warning signs, <https://www.cdc.gov/hearher/>

Provenance

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